WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 1

Psychotherapy Sprint

Complete High-Yield Review — Every Modality in One Document

All Modalities · Defence Mechanisms · CBT · DBT · Schema Therapy · Gottman · Yalom · Landmark Trials · Disorder Map

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Deep Study

Psychotherapy Sprint — Weave Psychotherapy Vol. 1

D1: Psychotherapy Sprint -- Complete High-Yield Review

KEY INSIGHT

Purpose: Single-document comprehensive review of all psychotherapy for MD Psychiatry exams

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1. FOUNDATIONS

1.1 Common Factors

The single most important concept in psychotherapy research: specific techniques account for far less outcome variance than the therapeutic relationship and patient factors.

Model	Key Factors	Core Claim
Rosenzweig (1936)	Common factors across therapies	"Everybody has won, and all must have prizes" (Dodo Bird)
Frank & Frank (1991)	Confiding relationship, healing setting, rationale/myth, ritual/procedure	Morale restoration is the mechanism
Lambert (1992)	Four outcome factors	See pie chart below
Wampold (2001, 2015)	Alliance, expectations, specific ingredients	Therapist effects (5–9%) > treatment differences

Lambert's Outcome Variance Breakdown

Factor	% of Outcome	What It Includes
Extratherapeutic	40%	Patient variables, life events, social support, spontaneous remission
Therapeutic relationship	30%	Alliance, empathy, warmth, acceptance
Expectancy/placebo	15%	Hope, credibility of rationale
Model/technique	15%	Specific therapeutic interventions

EXAM PEARL

Relationship + extratherapeutic = 70% of variance. Technique = only 15%. This is the foundation of the common factors argument. Know Lambert's 40/30/15/15 split cold.

KEY INSIGHT

MNEMONIC -- PRESS: Wolberg's nonspecific factors: **P**lacebo, **R**elationship, **E**motional catharsis, **S**uggestion, **S**ocial/group dynamics.

1.2 Therapeutic Alliance (Bordin, 1979)

The working alliance has three components:

Component	Definition
Goals	Mutually agreed targets of therapy
Tasks	Agreed therapeutic activities (exposure, free association, homework)
Bond	Quality of personal attachment -- trust, liking, respect

Key evidence:

- Alliance-outcome correlation: r ~ 0.275 (~7.5% variance) -- Horvath et al. (2011), N > 14,000
- Early alliance (sessions 3–5) is most predictive
- Repaired ruptures lead to BETTER outcomes than no ruptures (Safran & Muran, 2000)
- Two rupture types: withdrawal (disengagement) and confrontation (direct challenge)

EXAM PEARL

Safran & Muran (2000): repaired alliance ruptures predict better outcomes than no rupture at all. The rupture-repair cycle is itself therapeutic.

1.3 Rogers' Core Conditions (1957)

Six necessary and sufficient conditions for therapeutic change. The therapist-offered triad:

1. **Congruence/Genuineness** -- therapist is authentic and integrated
2. **Unconditional Positive Regard** -- non-judgmental acceptance
3. **Empathic Understanding** -- accurately perceiving the client's internal frame of reference

KEY INSIGHT

MNEMONIC -- EUG (or CUE): Empathy, Unconditional positive regard, Genuineness.

EXAM PEARL

Truax & Carkhuff (1967): patients of therapists with HIGH EUG improved. Patients of therapists with LOW EUG DETERIORATED. The therapist can be toxic.

1.4 Wolberg's Classification

Level	Goal	Examples
Supportive	Symptom relief	Guidance, reassurance, medication management
Reeducative	Attitude/behaviour change	CBT, behaviour therapy, counselling
Reconstructive	Personality restructuring	Psychoanalysis, psychodynamic therapy, existential analysis

1.5 Ethics Essentials

- **Absolute prohibition:** Sexual contact with patients (current or former within specified period)
- **Boundary crossing vs. violation:** Context, intent, and impact determine classification
- **Tarasoff duty:** Duty to warn/protect identifiable third parties from imminent danger
- **Mandatory reporting:** Danger to self, danger to others, child abuse, elder abuse

2. THE PSYCHIATRIC INTERVIEW

2.1 Shea's Validity Techniques

Technique	What It Does	Example
Behavioural incident	Elicits specific facts ("verbal videotape")	"What happened next?"
Shame attenuation	Makes positive answer ego-syntonic	"Are you pretty good at holding your liquor?"
Gentle assumption	Assumes behaviour is occurring	"What other ways have you thought of killing yourself?"
Symptom amplification	Sets upper limit high	"How many fights -- 20, 30, 40?"
Denial of the specific	Asks about specific items, not categories	"Coke?" "Speed?" "Marijuana?"
Normalisation	References what others experience	"Many people in your situation have thoughts of..."
Exaggeration	Humorously overstates	Sparingly; timing-dependent
Induction to bragging	Compliment then inquire	"You clearly don't take any shit -- how many fights?"

EXAM PEARL

Behavioural incident is the foundation of all other techniques. It creates a "verbal videotape" of what actually happened, not the patient's interpretation. Always start here.

2.2 CASE Approach (Chronological Assessment of Suicide Events)

Stage	Region	Key Technique
0	Set the platform	Enter through depression, psychosis, or crisis gate
1	Presenting events	Verbal videotape via behavioural incidents
2	Recent events (6–8 weeks)	Gentle assumption + denial of the specific
3	Past events	Most serious attempt, number, recency
4	Immediate events	"Right now, are you having thoughts of killing yourself?"

CLINICAL ANCHOR

Safety contracting is an assessment tool, NOT a guarantee. Always watch nonverbals and document quality of the contract. Never accept first "no" without further exploration.

3. PSYCHODYNAMIC THERAPY

3.1 Core Concepts

Definition: Therapy involving attention to the therapist-patient interaction, with interpretation of transference and resistance in a two-person field.

Seven basic principles (Gabbard): Much of mental life is unconscious; childhood + genetics shape the adult; transference is primary data; countertransference reveals patient's impact; resistance is a major focus; symptoms serve multiple functions; therapy aims for authenticity.

KEY INSIGHT

MNEMONIC -- SURMCRA: Subjectivity, Unconscious, Resistance, Multiple functions, Childhood + genetics, Response of therapist (CT), Attribution to therapist (transference).

3.2 Defence Mechanisms (Vaillant Hierarchy)

Primitive (Immature) Defences

Defence	Definition	Example
Splitting	All-good/all-bad compartmentalisation	"My last doctor was amazing; you're useless"
Projective identification	Project + pressure target to enact it	Patient's helplessness makes therapist overprotective
Projection	Attribute own impulses to others	"Everyone at work hates me" (disowned hostility)
Denial	Disregard external reality	Alcoholic: "I can stop anytime"
Dissociation	Disrupt identity/memory/consciousness	No memory of abuse
Idealisation/Devaluation	Exaggerated positive or negative attribution	"Only you understand me" / "Therapy is a waste"
Acting out	Impulsive enactment to avoid affect	Unprotected sex after painful session
Somatisation	Emotional pain becomes physical symptoms	Back pain worsens when mother visits
Regression	Return to earlier developmental phase	Adult curls into foetal position

KEY INSIGHT

MNEMONIC -- SPA-DIRSS: Splitting, Projective identification, Acting out, Denial, Idealisation/devaluation, Regression, Somatisation, Schizoid fantasy.

Neurotic (Higher-Level) Defences

Defence	Definition	Example
Repression	Expel unacceptable ideas from consciousness	Cannot recall childhood abuse
Displacement	Shift feelings to less threatening target	Angry at boss, kicks dog
Reaction formation	Transform wish into its opposite	Hatred becomes excessive concern
Intellectualisation	Abstract ideation to avoid feelings	Discusses parent's death via statistics
Isolation of affect	Separate idea from emotion	Describes abuse calmly, no affect
Rationalisation	Plausible but untrue justification	"I hit him because he needs to learn"
Undoing	Negate implications by opposite action	OCD handwashing after "dirty" thoughts

Mature Defences

Defence	Definition	Example
Sublimation	Channel aims into valued activity	Aggression becomes competitive sport
Humour	Find comic elements; distance without denial	Terminal patient jokes about taxes
Suppression	Consciously postpone attending to feeling	"I'll deal with this after the meeting"
Altruism	Constructive service to others	Suicide survivor volunteers at crisis line
Anticipation	Realistic planning for future	Studying months before exams

EXAM PEARL

Splitting + projective identification = borderline level. Repression-based defences = neurotic level. Mature defences = healthy. The Grant Study (Vaillant, 1977) showed mature-defence users had better health, marriages, and careers over 75 years.

3.3 Transference

Type	Description	Management
Positive	Warm, trusting	Foster; do NOT interpret unless it becomes resistance
Negative	Hostile, contemptuous	Interpret when it impedes work
Erotic	Sexual/romantic feelings	Interpret the function; maintain boundaries
Erotised	Fixed, demanding, insists on gratification	Limit-set; associated with severe pathology

Triangle of Insight (Menninger/Malan): Links transference (T), current relationships (C), and past relationships (P). A complete interpretation connects all three points.

> EXAM PEARL
>
> Concordant countertransference = therapist identifies with patient's self (empathy). Complementary countertransference = therapist identifies with patient's object (enacts role of abuser/parent). Complementary CT is more diagnostically informative and more dangerous if unrecognised.

3.4 Supportive vs Expressive

Feature	Supportive	Expressive
Goal	Stabilisation, functioning	Structural personality change
Defences	Reinforce	Interpret
Transference	Foster positive; don't interpret	Analyse as primary data
Regression	Prevent	Tolerate/utilise
Frequency	1×/week or less	2–3×/week
Patient	Borderline-level organisation	Neurotic-level organisation

> EXAM PEARL
>
> Wallerstein (1986) Menninger Project: supportive therapy produced as much structural change as expressive therapy over 30 years. The relationship is mutative regardless of interpretation.

3.5 Key Psychodynamic Techniques

Intervention continuum (most expressive to most supportive): Interpretation -> Observation -> Confrontation -> Clarification -> Encouragement to elaborate -> Empathic validation -> Psychoeducation -> Advice/Praise.

MNEMONIC

"I Often Confront Clearly, Encouraging Empathy And Praise."

3.6 Evidence

Study	Key Finding
Shedler (2010)	Effect size d = 0.97; **sleeper effect** -- gains INCREASE at follow-up
Leichsenring & Rabung (2008)	LTPP superior for complex disorders (d = 1.8 overall effect; d = 0.96 vs controls)
Steinert et al. (2017)	Psychodynamic vs CBT: g = -0.07 (no difference)

EXAM PEARL

The "sleeper effect" (Shedler 2010): psychodynamic therapy effect sizes INCREASE at follow-up because patients internalise a reflective process. CBT gains are maintained but typically do not increase. This is the strongest argument for psychodynamic therapy's unique contribution.

4. COGNITIVE BEHAVIOUR THERAPY

4.1 The Cognitive Model

Founder: Aaron T. Beck (1960s), originally a psychoanalyst.

Core premise: It is not the situation itself that determines feelings and behaviour, but how the person construes that situation.

Therapeutic stance: Collaborative empiricism -- therapist and client as co-investigators.

Three Levels of Cognition

Level	Characteristics	Modifiability
Automatic thoughts	Situation-specific, brief, spontaneous	Easiest
Intermediate beliefs	Rules, attitudes, conditional assumptions	Moderate
Core beliefs (schemas)	Global, absolute, rigid	Hardest

Three Categories of Negative Core Beliefs

Category	Theme	Example
Helplessness	I can't cope	"I am incompetent, powerless, a failure"
Unlovability	I can't be loved	"I am unlikeable, unwanted, bound to be rejected"
Worthlessness	I am bad	"I am worthless, dangerous, don't deserve to live"

KEY INSIGHT

MNEMONIC -- Three H's: Helplessness, Heartless world (Unlovability), Hopeless character (Worthlessness).

4.2 Cognitive Distortions (Full List)

Distortion	Definition	One-Line Example
All-or-nothing thinking	Only two categories	"If I'm not perfect, I'm a total failure"
Catastrophizing	Predicting the worst	"I'll be so upset I won't function at all"
Disqualifying the positive	Dismissing positives	"That success was just luck"
Emotional reasoning	Feelings as evidence	"I feel like a failure, so I must be one"
Labeling	Fixed global label	"I'm a loser"
Magnification/minimization	Inflating negatives, shrinking positives	"One bad rating proves I'm inadequate"
Mental filter	Attending to one negative	"One low score means I'm doing terribly"
Mind reading	Assuming others' thoughts	"He thinks I'm incompetent"
Overgeneralization	Sweeping conclusions	"I felt awkward; I'll never make friends"
Personalization	Attributing others' behaviour to self	"The repairman was rude because of me"
Should/must statements	Rigid rules	"I should always do my best"
Tunnel vision	Seeing only negatives	"My boss can't do anything right"

KEY INSIGHT

MNEMONIC -- FLAME MOST: Fortune-telling (catastrophizing), Labeling, All-or-nothing, Mind reading, Emotional reasoning -- Magnification, Overgeneralization, Should statements, Tunnel vision. (Covers 9/12; add mental filter, personalization, disqualifying the positive.)

EXAM PEARL

Emotional reasoning is the distortion most commonly missed by trainees. Especially prominent in anxiety and BPD.

4.3 CBT Session Structure

Phase	Steps
Initial	Mood check, set agenda, update + review Action Plan, prioritise
Middle	Work on agenda items (summarise, intervene, set Action Plan)
End	Summarise session, review Action Plan, elicit feedback

KEY INSIGHT

MNEMONIC -- MADAM-SURF: Mood, Agenda, Discuss update, Agenda prioritise, Middle work -- Summarise, Update AP, Review feedback, Finish.

4.4 Key CBT Techniques

- **Socratic questioning:** Guided discovery through strategic questions
- **Thought record (DTR):** 6–column worksheet -- situation, AT, belief rating, emotion, adaptive response, outcome
- **Behavioural experiments:** Collaboratively test negative predictions
- **Activity scheduling:** Mastery (M) and Pleasure (P) ratings
- **Downward arrow:** "If that were true, what would it mean about you?" -- reaches core beliefs in 3–5 steps

4.5 Disorder-Specific CBT Models

Disorder	Model/Authors	Key Mechanism	Key Intervention
Depression	Beck's cognitive triad	Negative views of self, world, future	BA + cognitive restructuring
Panic disorder	Clark (1986)	Catastrophic misinterpretation of bodily sensations	Interoceptive exposure
Social anxiety	Clark & Wells (1995)	Self-focused attention, safety behaviours, post-event rumination	Video feedback, attention training
OCD	Salkovskis (1985)	Inflated responsibility, thought-action fusion	ERP + cognitive restructuring
PTSD	Ehlers & Clark (2000)	Negative appraisals + fragmented memory	Reliving with cognitive restructuring
Eating disorders	Fairburn (2008) -- CBT-E	Over-evaluation of shape/weight	Regular eating, broadening self-evaluation
Insomnia	Morin -- CBT-I	Dysfunctional sleep beliefs	Sleep restriction, stimulus control
Psychosis	CT-R (Beck et al.)	Defeatist performance beliefs	Aspirations-based goal setting

EXAM PEARL

Clark's (1986) cognitive model of panic is the most commonly examined disorder-specific CBT model. Vicious circle: bodily sensation -> catastrophic misinterpretation -> anxiety -> more sensations. Interoceptive exposure breaks the cycle. 80–90% panic-free rates.

4.6 PD Cognitive Profiles (Beck)

PD	Core Belief	Compensatory Strategy
Avoidant	"I am inadequate"	Avoid evaluative situations
Dependent	"I need help to survive"	Cultivate dependent relationships
OCPD	"I must not err"	Apply rules rigidly
Paranoid	"People are dangerous"	Vigilance, counterattack
Narcissistic	"I deserve special treatment"	Self-aggrandizement
Antisocial	"Rules are for others"	Exploit, attack
BPD	"I will be abandoned / I am bad"	Clinging, splitting, self-harm

KEY INSIGHT

MNEMONIC -- PD belief anchors by cluster: Cluster A = "Others are dangerous/intrusive." Cluster B = "I am special/entitled/abandoned." Cluster C = "I am inadequate/helpless/must not err."

4.7 CBT Evidence

EXAM PEARL

CBT's strongest evidence (largest effect sizes) is for panic disorder, social anxiety disorder, and OCD. For depression, CBT's unique advantage is relapse prevention -- Hollon et al. (2005): relapse 31% after prior CT vs 76% after medication withdrawal.

5. BEHAVIOUR THERAPY

5.1 Classical vs Operant Conditioning

Feature	Classical	Operant
Pioneer	Pavlov (1897)	Skinner (1938)
Learning	Stimulus-stimulus association	Behaviour-consequence association
Response type	Involuntary (salivation, fear)	Voluntary (pressing lever, avoidance)
Extinction	CS without US -> CR weakens	No reinforcement -> behaviour weakens
Clinical use	Phobias, PTSD, conditioned drug cues	Avoidance, substance use, token economy

5.2 Reinforcement Grid

Add Stimulus (+)	Remove Stimulus (-)	
Behaviour increases	**Positive reinforcement** (praise -> more homework)	**Negative reinforcement** (anxiolytic removes anxiety -> more pill-taking)
Behaviour decreases	**Positive punishment** (scold -> less misbehaviour)	**Negative punishment** (remove phone -> fewer outbursts)

EXAM PEARL

Negative reinforcement is NOT punishment. It INCREASES behaviour by removing something aversive (e.g., avoidance reduces anxiety, so avoidance increases). This is the mechanism maintaining all avoidance behaviour.

5.3 Schedules of Reinforcement

Schedule	Extinction Resistance	Clinical Example
Fixed Ratio	Fast extinction	Token economy (5 tasks = 1 token)
Variable Ratio	**Slowest (most resistant)**	**Gambling; reassurance-seeking**
Fixed Interval	Moderate	Medication schedules
Variable Interval	Slow	Random check-ins

EXAM PEARL

VR = most resistant to extinction. This is why gambling and intermittent reinforcement create the strongest habits.

5.4 Exposure Therapy Types

Type	Method	Primary Indication
In-vivo	Real-world confrontation	Specific phobias, agoraphobia
Imaginal	Mental imagery	PTSD (Prolonged Exposure)
Interoceptive	Induce feared body sensations	Panic disorder
Virtual reality	Computer simulation	Flying phobia, heights
Flooding	Maximum-intensity prolonged	Rapid reduction; high dropout
Graded	Systematic hierarchy	Standard for most anxiety
ERP	Exposure + ritual prevention	OCD (gold standard; 60–70% response)

5.5 Systematic Desensitisation (Wolpe, 1958)

Steps: (1) Relaxation training (PMR), (2) Build anxiety hierarchy (0–100 SUDs), (3) Pair relaxation + imagery from lowest, (4) Progress up hierarchy, (5) In-vivo transfer.

Mechanism: Reciprocal inhibition -- relaxation inhibits anxiety. Now largely replaced by direct exposure therapy.

5.6 Behavioural Activation

Model: Depression = low response-contingent positive reinforcement (Lewinsohn, 1974). Treatment = increase valued activities. Action BEFORE motivation.

Key technique -- TRAP to TRAC: Trigger -> Response -> Avoidance Pattern -> replace with -> Alternative Coping.

> EXAM PEARL
>
> Dimidjian et al. (2006): BA = paroxetine > cognitive therapy for SEVERE depression. BA is simpler, cheaper, and can be delivered by non-specialists. MANAS trial (Patel 2010, Lancet): lay-delivered BA effective in India.

5.7 Key Behavioural Concepts

- **Extinction is NOT erasure.** New inhibitory memory competes with original fear. Explains spontaneous recovery, renewal, and reinstatement.
- **Inhibitory learning model (Craske):** Maximise expectancy violation, vary contexts, remove safety signals, deepen extinction.
- **Self-efficacy (Bandura):** Strongest predictor of whether a patient will attempt a feared task. Four sources: mastery experiences (strongest), vicarious experience, verbal persuasion, physiological states.
- **Safety behaviours maintain fear structures.** Must be eliminated during exposure.

> CLINICAL ANCHOR
>
> Mary Cover Jones (1924) = "mother of behaviour therapy" -- first systematic fear elimination (Little Peter). Watson & Rayner (1920) = Little Albert (fear conditioning).

6. THIRD-WAVE THERAPIES

6.1 What Makes Third Wave Different

Wave	Era	Core Idea
First	1950s-1970s	Modify observable behaviour
Second	1970s-1990s	Change distorted cognitions
Third	**1990s-present**	**Change relationship to thoughts; acceptance + mindfulness + values**

6.2 DBT (Linehan, 1993)

Biosocial Model

Chronic emotional dysregulation = biological vulnerability (high sensitivity + high reactivity + slow return to baseline) x invalidating environment. The model is transactional and de-shaming.

Target Hierarchy (governs every session)

1. **Life-threatening behaviours** (always first)
2. **Therapy-interfering behaviours**
3. **Quality-of-life-interfering behaviours**
4. **Increasing skills**

> EXAM PEARL
>
> The DBT target hierarchy is absolute. If suicidal behaviour occurred this week, it is addressed before everything else, regardless of what the patient wants to discuss. This is one of the most heavily examined DBT concepts.

Four Modes of DBT

1. Individual therapy (applies target hierarchy)
2. Skills group (teaches skills)
3. Phone coaching (generalisation to daily life)
4. Therapist consultation team (treats the therapists)

DBT Skills by Module

Core Mindfulness:

- Wise Mind (synthesis of Emotional + Reasonable Mind)
- "What" skills: Observe, Describe, Participate
- "How" skills: Non-judgmentally, One-mindfully, Effectively

Distress Tolerance -- Crisis Survival:

Skill	Components
TIPP	Temperature (dive reflex), Intense exercise, Paced breathing, Progressive relaxation
STOP	Stop, Take a step back, Observe, Proceed mindfully
ACCEPTS	Activities, Contributing, Comparisons, Emotions, Pushing away, Thoughts, Sensations
IMPROVE	Imagery, Meaning, Prayer, Relaxation, One thing, Vacation, Encouragement

Distress Tolerance -- Reality Acceptance:

- Radical Acceptance: Pain + Non-acceptance = Suffering
- Turning the Mind: ongoing choice to accept
- Willingness vs. Willfulness

Emotion Regulation:

Skill	Description
ABC PLEASE	Accumulate positives, Build mastery, Cope ahead + PhysicaL illness, Eating, Avoid substances, Sleep, Exercise
Check the Facts	Is the emotion fitting the facts?
Opposite Action	Act opposite to urge when emotion does not fit facts
Problem Solving	When emotion fits facts, solve the problem

Opposite Action Quick Card:

Emotion	Urge	Opposite
Fear	Avoid	Approach
Anger	Attack	Avoid; empathy
Sadness	Withdraw	Get active
Shame (unjustified)	Hide	Go public

Interpersonal Effectiveness:

Skill	Goal	Components
DEAR MAN	Get what you want	Describe, Express, Assert, Reinforce, Mindful, Appear confident, Negotiate
GIVE	Maintain relationship	Gentle, Interested, Validate, Easy manner
FAST	Maintain self-respect	Fair, no Apologies, Stick to values, Truthful
THINK	Reduce hostility (adolescent)	Think, Have empathy, Interpretations, Notice, Kindness

Six Validation Levels

Level	Description
1	Unbiased listening
2	Accurate reflection
3	Mind-reading (articulating unspoken experiences)
4	Validation by past history
5	**Validation by current context (heart of DBT validation)**
6	**Radical genuineness (treating person as capable equal)**

EXAM PEARL

Level 5 validation is the most important clinically -- normalising the response given current circumstances. Level 6 is the highest and most challenging.

6.3 Schema Therapy (Young, 1990/2003)

18 Early Maladaptive Schemas (EMSs)

Domain	Schemas
I. Disconnection & Rejection (5)	Abandonment, Mistrust/Abuse, Emotional Deprivation, Defectiveness/Shame, Social Isolation
II. Impaired Autonomy (4)	Dependence/Incompetence, Vulnerability to Harm, Enmeshment, Failure
III. Impaired Limits (2)	Entitlement/Grandiosity, Insufficient Self-Control
IV. Other-Directedness (3)	Subjugation, Self-Sacrifice, Approval-Seeking
V. Overvigilance & Inhibition (4)	Negativity/Pessimism, Emotional Inhibition, Unrelenting Standards, Punitiveness

Domain distribution: 5–4–2–3–4 (memorise this).

Five core emotional needs: Secure attachment, Autonomy/competence, Freedom to express needs, Spontaneity/play, Realistic limits.

Three coping styles: Surrender (Freeze), Avoidance (Flight), Overcompensation (Fight).

Schema Modes

Category	Modes
Child	Vulnerable Child, Angry Child, Impulsive/Undisciplined Child, Happy Child
Coping	Compliant Surrenderer, Detached Protector, Overcompensator
Parent	Punitive Parent, Demanding Parent
Healthy	Healthy Adult

BPD Mode Model (4 core modes): Abandoned/abused child, Angry/impulsive child, Punitive parent (signature BPD mode), Detached protector.

Key ST Techniques

- **Imagery rescripting:** Access childhood memory via affect bridge -> helping figure changes the outcome -> not reliving trauma, but rescripting it
- **Chair work:** Dialogue between modes; battle the Punitive Parent
- **Limited reparenting:** Provide within boundaries what the patient needed from parents
- **Empathic confrontation:** Validate experience while confronting maladaptive patterns

> EXAM PEARL
>
> Arntz et al. (2007): imagery rescripting was superior to imaginal exposure on guilt, shame, and anger in PTSD. IR is not exposure -- the memory is actively changed, not just habituated to.

> EXAM PEARL
>
> Giesen-Bloo et al. (2006): ST recovery 45.5% vs TFP 24.4% for BPD; ST dropout 26.7% vs TFP 50.6%. The only head-to-head RCT showing clear superiority. Note: sedatives/benzodiazepines significantly reduced ST effectiveness (blocks emotional processing for IR).

6.4 ACT (Hayes, 1999)

Hexaflex -- Six Processes of Psychological Flexibility

Process	Inflexible Pole	Flexible Pole
Acceptance	Experiential avoidance	Willingness to have unwanted experiences
Defusion	Cognitive fusion	Seeing thoughts as thoughts, not facts
Present Moment	Past/future dominance	Here-and-now awareness
Self-as-Context	Conceptualised self	Observing self ("I am the sky, not the weather")
Values	Lack of clarity	Chosen life directions
Committed Action	Inaction/avoidance	Values-consistent behaviour

Two clusters: Mindfulness/acceptance (processes 1–4) and Commitment/change (processes 5–6).

Core distinction from CBT: ACT does not aim to change thought content. It changes the relationship to thoughts. Symptom reduction is a byproduct, not a goal.

Key techniques: Creative hopelessness, "I'm having the thought that...", leaves on a stream, values clarification.

> EXAM PEARL
>
> A-Tjak et al. (2015): ACT g = 0.57 vs controls; equivalent to CBT (g = 0.06, ns difference). ACT works, but not better than CBT.

6.5 MBCT (Segal, Williams & Teasdale, 2002)

8–session group programme for depression relapse prevention.

Session	Theme
1	Automatic pilot (raisin exercise, body scan)
2	Dealing with barriers
3	Mindfulness of breath
4	Staying present ("thoughts are not facts")
5	Allowing/letting be
6	Thoughts are not facts
7	Self-care (nourishing vs depleting activities)
8	Maintaining practice

3-Minute Breathing Space: Minute 1 = Awareness ("What is my experience right now?"), Minute 2 = Gathering (narrow to breath), Minute 3 = Expanding (whole body).

Mechanism: Doing mode (discrepancy monitoring -> rumination) becomes Being mode (present-focused acceptance).

> EXAM PEARL
>
> MBCT reduces relapse ONLY for patients with 3+ episodes (Teasdale, 2000: 37% vs 66%). NO benefit for 1–2 episodes. This is because differential activation (mild sadness reactivating full depressive cognition) only develops after multiple episodes.

> EXAM PEARL
>
> Kuyken et al. (2015): MBCT non-inferior to maintenance antidepressants over 24 months.

7. HUMANISTIC-EXISTENTIAL THERAPIES

7.1 Rogers' Person-Centred Therapy

- **Actualising tendency:** Innate drive toward growth
- **Conditions of worth:** "I am lovable only if..." -- distorts organismic valuing
- **Incongruence:** Gap between self-concept and experience = distress
- **Paradoxical theory of change (Beisser):** Change happens when you fully become what you are, not what you try to be

7.2 Gestalt Therapy (Perls)

Contact Boundary Disturbances

Disturbance	Definition
Introjection	Swallowing beliefs without questioning
Projection	Attributing disowned parts to others
Retroflection	Doing to self what one wants to do to/get from others
Deflection	Avoiding contact via humour, abstraction
Confluence	Merging; no self-other boundary

> KEY INSIGHT
>
> **MNEMONIC -- I-PRDC:** Introjection, Projection, Retroflection, Deflection, Confluence.

Key Gestalt Techniques

- **Empty chair:** Talk to imagined absent person (resolves unfinished business)
- **Two-chair (hot seat):** Alternate between conflicting self-parts (integrates polarities)

- **Exaggeration:** Amplify gesture/statement for awareness
- **Dream work:** Become every dream element (NOT Freudian)

> EXAM PEARL
>
> Empty chair = unfinished business with others. Two-chair = internal conflict between self-parts. This distinction is commonly tested.

7.3 Motivational Interviewing (Miller & Rollnick)

MI Spirit -- PACE

Partnership, Acceptance, Compassion, Evocation

Core Skills -- OARS

Open questions, Affirmations, Reflections, Summaries

Change Talk -- DARN-CAT

- **Preparatory:** Desire, Ability, Reasons, Need
- **Mobilising:** Commitment, Activation, Taking steps

Key concepts:

- "Resistance" replaced by **sustain talk** (content) and **discord** (process/relational friction)
- Readiness ruler: "0–10, how important?" then "Why X and not lower?" (evokes change talk)

> CLINICAL ANCHOR
>
> MI is a communication style, not a full therapy. It resolves ambivalence by eliciting the patient's own arguments for change. Works across substance use and any behaviour change context.

7.4 Stages of Change (Prochaska & DiClemente, 1983)

Stage	Therapeutic Task
Precontemplation	Raise awareness
Contemplation	Explore ambivalence
Preparation	Strengthen commitment
Action	Support self-efficacy
Maintenance	Relapse prevention
Relapse	Normalise; re-engage

EXAM PEARL

The Transtheoretical Model was developed INDEPENDENTLY from MI. They are separate frameworks that are often used together.

7.5 Yalom's Four Ultimate Concerns

Concern	Core Conflict	Defence
Death	Permanence wish vs mortality	Specialness, ultimate rescuer
Freedom	Structure wish vs groundlessness	Blame, compulsive compliance
Isolation	Merger wish vs unbridgeable gap	Fusion, compulsive socialising
Meaninglessness	Meaning need vs cosmic indifference	Crusadism, compulsive activity

KEY INSIGHT

MNEMONIC -- DFIM: Death, Freedom, Isolation, Meaninglessness -- "Don't Forget, I Matter."

7.6 Logotherapy (Frankl)

- **Will to meaning:** Primary human drive (not pleasure or power)
- **Three pathways to meaning:** Creative values (what we give), experiential values (what we receive), attitudinal values (stance toward suffering)
- **Paradoxical intention:** Intend/wish for the feared outcome -> breaks anticipatory anxiety via humour
- **Dereflection:** Redirect attention from hyper-self-monitoring toward engagement

8. SPECIALIZED MODALITIES

8.1 IPT (Klerman, 1984)

Four Problem Areas

Problem Area	Key Techniques
Grief	Facilitate mourning; reconstruct relationship (positive + negative)
Role disputes	Clarify expectations; communication analysis; stages: renegotiation/impasse/dissolution
Role transitions	Mourn old role; develop mastery in new role
Interpersonal deficits	Use therapeutic relationship as model; encourage social risk-taking

Three phases: Phase 1 (sessions 1–3): sick role, interpersonal inventory, identify problem area. Phase 2 (sessions 4–12): work on problem area. Phase 3 (sessions 13–16): termination as role transition.

EXAM PEARL

The "sick role" (Parsons) in IPT legitimises symptoms, reduces self-blame, and obligates the patient to work toward recovery. Assigned in Phase 1.

CLINICAL ANCHOR

Elkin et al. (1989) NIMH TDCRP: IPT equivalent to imipramine for moderate-severe depression. IPT is also effective for bulimia (Fairburn 1993: equivalent to CBT at 12–month follow-up) and perinatal depression.

8.2 EMDR (Shapiro, 1989/2018)

Eight Phases

Phase	Name	Key Activity
1	History & planning	Identify targets
2	Preparation	Safe place, explain procedure
3	Assessment	Image + NC + PC + VOC (1–7) + SUDS (0–10) + body location
4	Desensitization	BLS until SUDS = 0
5	Installation	Strengthen PC until VOC = 7
6	Body scan	Clear somatic residue
7	Closure	Stabilise; safe place if incomplete
8	Re-evaluation	Next session check

AIP model: Trauma overwhelms innate processing -> stored in state-specific form -> triggered by current stimuli -> symptoms. EMDR resumes adaptive processing.

> EXAM PEARL
>
> EMDR is equivalent to TF-CBT for PTSD (WHO, NICE, APA first-line). Mechanism of BLS debated: working memory taxation (van den Hout), orienting response, REM analogy. Key scales: SUDS (0–10, disturbance), VOC (1–7, belief validity).

8.3 MBT (Bateman & Fonagy, 1999)

Mentalization: Understanding behaviour in terms of underlying mental states -- "holding mind in mind."

Pre-mentalizing modes:

- **Psychic equivalence:** Inner = outer ("I feel it, so it must be true")
- **Teleological mode:** Only actions count ("If you cared, you'd extend the session")
- **Pretend mode:** Intellectualised talk disconnected from affect

Key techniques: Not-knowing stance, stop and rewind, affect focus.

> EXAM PEARL
>
> Bateman & Fonagy (2008) 8–year follow-up: only 13% still met BPD criteria vs 87% control. The longest follow-up data for any BPD treatment.

8.4 TFP (Kernberg)

Kernberg's Three Criteria for Personality Organisation:

Dimension	Neurotic	Borderline	Psychotic
Identity	Integrated	Diffuse	Fragmented
Defences	Mature (repression)	Primitive (splitting, PI)	Psychotic
Reality testing	Intact	Intact but lapses	Impaired

TFP technique hierarchy: Life threats -> treatment threats -> acting out -> contract violations -> narcissistic transference -> "as if" material.

Key technique chain: Clarification -> Confrontation -> Transference interpretation (the primary mutative intervention).

> EXAM PEARL
>
> Clarkin et al. (2007): TFP is the ONLY BPD treatment shown to change reflective function and attachment organisation.

8.5 EFT for Couples (Johnson, 1980s)

Three Stages, Nine Steps

Stage	Focus	Key Moment
1. De-escalation (Steps 1–4)	Identify negative cycle; access primary emotions; reframe: cycle is the enemy	Both partners see the pattern
2. Restructuring (Steps 5–7)	Promote identification with needs; facilitate bonding events	**Softening** -- blaming partner expresses vulnerability
3. Consolidation (Steps 8–9)	New solutions; consolidate secure cycle	Secure bond replaces old cycle

> EXAM PEARL
>
> The "softening" is the pivotal moment of EFT. The previously critical partner reaches from vulnerability (not anger), and the withdrawing partner responds with comfort. This is the corrective attachment experience.

> MNEMONIC
>
> EFT stages = "Stop, Open, Stay" -- Stop the cycle, Open to vulnerability, Stay connected.

8.6 Transactional Analysis (Berne)

Ego states: Parent (Critical/Nurturing), Adult (reality testing), Child (Free/Adapted).

Transaction types: Complementary (communication continues), Crossed (communication breaks down), Ulterior (hidden psychological message determines behaviour).

Games: Repetitive ulterior transactions with negative payoff. Examples: "Why Don't You... Yes But" (confirms "nobody can help me"), "If It Weren't For You" (avoids testing own capabilities).

8.7 Supportive Therapy

Techniques: Ventilation, reassurance, advice/guidance, praise, anticipatory guidance, clarification, suggestion, environmental intervention.

> EXAM PEARL
>
> Supportive psychotherapy is the most widely practised yet most undertaught modality. Every psychiatrist must be competent in it. De Jonghe et al. (2001): equivalent to psychodynamic therapy at 6 months.

9. GROUP THERAPY

9.1 Yalom's 11 Therapeutic Factors

#	Factor	Core Idea
1	Instillation of hope	Seeing others recover
2	Universality	"I am not alone"
3	Imparting information	Psychoeducation, advice
4	Altruism	Helping others heals the helper
5	Corrective recapitulation of primary family	Rework early family dynamics
6	Development of socialising techniques	Learn social skills via feedback
7	Imitative behaviour	Model after therapist/members
8	**Interpersonal learning**	**Group as social microcosm** (most distinctive)
9	**Group cohesiveness**	**Belonging and acceptance** (most foundational)
10	Catharsis	Safe emotional expression
11	Existential factors	Death, isolation, freedom, meaning

EXAM PEARL

Group cohesiveness is to group therapy what the therapeutic alliance is to individual therapy -- the foundational condition without which no other factor operates.

9.2 Group Stages

Tuckman Stage	Key Process	Dropout Risk
Forming	Orientation, dependency	Low
Storming	Conflict, dominance struggles	**Highest**
Norming	Cohesion, trust	Low
Performing	Interpersonal learning, genuine intimacy	Minimal
Adjourning	Mourning, consolidation	N/A

> EXAM PEARL
>
> Most premature dropouts occur during the storming/transition phase. Members who survive this phase show the greatest therapeutic gains.

9.3 Bion's Basic Assumptions

Three unconscious group processes that interfere with rational task:

1. **Dependency** -- group looks to leader as omnipotent figure
2. **Fight-flight** -- group unites against perceived enemy or avoids task
3. **Pairing** -- two members form dyad while group watches with messianic hope

9.4 Key Group Therapy Facts

- Optimal size: 7–8 members (process group)
- Top contraindication: severe antisocial personality
- Foulkes: group analysis; therapist as "conductor"
- Pratt (1905): "father of group therapy" -- TB patients

10. FAMILY THERAPY

10.1 Schools Comparison

School	Founder	Core Concept	Key Technique
Structural	Minuchin	Boundaries, hierarchy, subsystems	Joining, enactment, restructuring
Strategic	Haley, MRI	Attempted solutions maintain problems	Directives, paradox, reframing
Systemic/Milan	Selvini Palazzoli	Circular causality, neutrality	Circular questioning, positive connotation
Bowenian	Bowen	Differentiation of self, triangles	Genogram, coaching, I-position
Experiential	Satir, Whitaker	Emotional experience, communication	Family sculpting, communication stances
Narrative	White & Epston	Dominant narratives, externalisation	Externalising, unique outcomes
Solution-Focused	de Shazer, Berg	Exceptions, strengths	Miracle question, scaling

10.2 Key Family Therapy Concepts

- **Systems theory:** Family = more than sum of parts
- **Circular causality:** Cause and effect are reciprocal
- **Homeostasis:** System resists change; improvement in IP may trigger deterioration in another

- **Identified patient (IP):** The symptom-bearer
- **Boundaries:** Clear (healthy), rigid (disengaged), diffuse (enmeshed)
- **Triangulation:** Two-person stress recruits a third to stabilise (Bowen's fundamental unit)
- **Double bind (Bateson):** Contradictory messages at different levels from which there is no escape
- **Differentiation of self (Bowen):** Capacity to maintain self while emotionally connected
- **Psychosomatic family (Minuchin):** Enmeshment + overprotectiveness + rigidity + poor conflict resolution

> EXAM PEARL
>
> Homeostasis explains why individual therapy gains collapse when a patient returns to their family. The system is invested in its current equilibrium -- even pathological equilibrium.

> CLINICAL ANCHOR
>
> Expressed Emotion: critical comments, hostility, emotional overinvolvement. High EE = ~50% 9–month schizophrenia relapse (vs ~20% low EE). Family intervention reduces relapse by ~50% (Leff/Pilling 2000/2002).

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11. COUPLES THERAPY

11.1 Gottman's Four Horsemen + Antidotes

Horseman	Antidote
Criticism (global character attack)	Gentle start-up: "I feel... about... I need..."
Contempt (sarcasm, superiority, disgust)	Fondness and admiration; daily appreciation
Defensiveness (counter-blame)	Accept responsibility
Stonewalling (emotional shutdown; 85% male)	Self-soothing: 20+ min break when HR > 100 bpm

> EXAM PEARL
>
> Contempt is the single best predictor of divorce. It differs from criticism in that it conveys superiority and moral disgust. Contempt also predicts infectious illness in the partner receiving it.

Key Gottman Numbers

- 5:1 ratio of positive to negative interactions in stable marriages
- Four Horsemen alone = 82% divorce prediction accuracy
- Four Horsemen + failed repairs = 90%+ accuracy
- 69% of couple problems are perpetual (dialogue, not resolution)
- 96% of conversations end on the same note they begin

11.2 Sound Relationship House

Level	Component
7 (Top)	Create Shared Meaning
6	Make Life Dreams Come True
5	Manage Conflict
4	Positive Sentiment Override
3	Turning Toward (emotional bank account)
2	Fondness & Admiration
1 (Base)	Love Maps
WALLS	Trust & Commitment

> CLINICAL ANCHOR
>
> The first three levels (Love Maps, Fondness/Admiration, Turning Toward) build the friendship system. Without friendship, conflict management fails -- repair attempts depend on PSO, which depends on friendship.

12. SPECIAL POPULATIONS

12.1 Child and Adolescent

Age	Recommended Modalities
0–2	Parent-infant psychotherapy, VIG, Circle of Security
2–7	Play therapy, PCIT, CBPT
7–11	CBT (Coping Cat), group CBT
12–17	CBT adapted, DBT-A, IPT-A

Adolescent DBT (Rathus & Miller)

- 5th module: Walking the Middle Path (dialectics, validation, behaviour change)
- Multifamily skills group
- 24–week duration
- Three adolescent dilemmas: leniency vs authoritarian, normalising pathological vs pathologising normative, forcing autonomy vs fostering dependence

> EXAM PEARL
>
> Mehlum et al. (2014): DBT-A superior to enhanced usual care for adolescent self-harm. The key RCT for adolescent DBT.

12.2 CBT for Psychosis (CBTp)

Key techniques: Normalising rationale (continuum model), shared formulation, peripheral questioning (start with less charged beliefs), beliefs about voices work (Chadwick & Birchwood), coping strategy enhancement.

Freeman's 6 maintenance factors for paranoia: Anxiety, worry, negative self-beliefs, sleep problems, reasoning biases (jumping to conclusions), safety behaviours.

> EXAM PEARL
>
> CBTp effect size is modest (d = 0.33–0.44, Morrison 2014) but clinically meaningful. It is an adjunct, never standalone. Family intervention reduces schizophrenia relapse by ~50%.

12.3 Cultural Formulation Interview (CFI)

Six domains: cultural definition of problem, cultural perceptions of cause, stressors/supports, role of cultural identity, past coping/help-seeking, current help-seeking.

Kleinman's 8 questions for explanatory models. **Bernal's 8 adaptation dimensions:** Language, Persons, Metaphors, Content, Concepts, Goals, Methods, Context.

12.4 Phase-Based Trauma Treatment

Phase	Goal	Interventions
1	Stabilisation	Grounding, distress tolerance, alliance
2	Processing	PE, CPT, EMDR, NET
3	Integration	Interpersonal work, identity rebuilding

Contraindications for Phase 2: Active suicidality, ongoing abuse, severe dissociation (DES >30), active substance dependence, insufficient affect regulation, acute psychosis.

12.5 Older Adults

Laidlaw's CCCF model adds four cohort-specific factors to standard CBT: Cohort beliefs, Role transitions, Intergenerational linkages, Socio-cultural context.

> KEY INSIGHT
>
> **MNEMONIC -- CRIS:** Cohort beliefs, Role transitions, Intergenerational linkages, Socio-cultural context.

13. DISORDER-SPECIFIC MAP

13.1 Master Disorder-Therapy Table

Disorder	1st Line Therapy	Key Trial
MDD (mild-moderate)	CBT, BA, IPT	Rush 1977; Elkin 1989
MDD (severe)	CBT + SSRI, IPT + SSRI	Cuijpers 2020
Persistent depressive disorder	CBT, IPT, CBASP	Keller 2000 (NEJM)
Treatment-resistant depression	CBT augmentation	Wiles 2013 (CoBalT)
GAD	CBT, Applied Relaxation	Borkovec 1995
Social anxiety disorder	CBT (Clark & Wells)	Clark 2006
Specific phobia	In-vivo graded exposure	Ost 1989
Panic disorder	CBT (Clark model)	Clark 1994, 1999
OCD	ERP (Salkovskis model)	Foa 2005
BDD	CBT (adapted) + SSRI	Veale 2010
PTSD	TF-CBT (PE, CPT), EMDR	Bisson 2007
Complex PTSD	Phase-based: STAIR + PE	Cloitre 2010
BPD	DBT, MBT, TFP, Schema Therapy	Linehan 2006; Giesen-Bloo 2006
NPD	Schema Therapy, Psychodynamic	--
AvPD	CBT, Schema Therapy	Bamelis 2014
OCPD	CBT (Beck CT-PD)	--
AN (adolescent)	FBT (Maudsley)	Lock 2010
AN (adult)	CBT-ED, MANTRA, SSCM	Zipfel 2014 (ANTOP)

Disorder	1st Line Therapy	Key Trial
BN	CBT-E (Fairburn)	Fairburn 2009
BED	CBT, guided self-help	Wilfley 2002
Alcohol use disorder	MI, CBT, TSF	Project MATCH 1997
Stimulant use disorder	CBT, Contingency management	--
Schizophrenia	CBTp + family intervention	Morrison 2014; Pilling 2002
Bipolar disorder	Psychoeducation + mood stabiliser	Colom 2003; Frank 2005
Adolescent depression	CBT + SSRI (combined)	TADS 2004
Adolescent self-harm	DBT-A	Mehlum 2014
Child anxiety	CBT (Coping Cat)	Kendall 1997
Child conduct disorder	Parent training; MST (adolescent)	Henggeler 1998
Couples distress	EFT-C, Gottman Method	Johnson 1999
Insomnia	CBT-I	Trauer 2015
Perinatal depression	CBT or IPT	O'Hara 2000

EXAM PEARL

For mild-moderate depression, no therapy is clearly superior -- the Dodo bird verdict applies. For severe depression, combination (therapy + medication) is consistently superior to either alone.

CLINICAL ANCHOR

Five things to remember: (1) Depression: CBT = IPT = BA; combo superior for severe. (2) Anxiety: each disorder has a specific CBT model -- know the model. (3) BPD: four treatments (DBT, MBT, TFP, ST). (4) Psychosis: CBTp is adjunct; family intervention halves relapse. (5) Children: FBT for AN, parent training for conduct, CBT + SSRI for depression.

13.2 BPD Treatments Head-to-Head

Feature	DBT	MBT	TFP	Schema Therapy
Theory	Biosocial	Attachment	Object relations	Cognitive-developmental
Core target	Emotion dysregulation	Mentalizing failure	Split object relations	EMSs + modes
Stance	Dialectical	Not-knowing, curious	Technically neutral	Limited reparenting
Key technique	Skills + chain analysis	Mentalizing the moment	Transference interpretation	IR + chair work
Landmark RCT	Linehan 2006	Bateman & Fonagy 1999	Clarkin 2007	Giesen-Bloo 2006
Dropout	~25%	~15–25%	~35–50%	~20–27%
NICE recommended	Yes	Yes	No	No

EXAM PEARL

DBT has most evidence and is best for actively suicidal patients. MBT has longest follow-up (8 years). TFP uniquely changes attachment organisation. ST has highest recovery rate and lowest dropout.

14. LANDMARK TRIALS

14.1 Top 20 Trials

Trial	Year	Finding
Rush et al.	1977	First RCT: CT superior to imipramine for depression
Elkin et al. (NIMH TDCRP)	1989	All active treatments equivalent (mild-moderate); imipramine better in severe
DeRubeis et al.	2005	CT = paroxetine for moderate-severe MDD (58% vs 58%) -- refuted TDCRP
Hollon et al.	2005	Prior CT relapse 31% vs medication withdrawal 76%
Dimidjian et al.	2006	BA = medication > cognitive therapy for severe depression
Teasdale et al.	2000	MBCT halved relapse for 3+ episodes (37% vs 66%)
Kuyken et al.	2015	MBCT non-inferior to maintenance antidepressants
Clark et al.	2003	CT > fluoxetine for social anxiety; d > 1.0
Foa et al.	2005	ERP = clomipramine for OCD
Bisson et al.	2007	TF-CBT and EMDR both first-line for PTSD
Linehan et al.	1991	First DBT RCT: reduced parasuicide in BPD

Trial	Year	Finding
Linehan et al.	2006	DBT halved suicide attempts vs expert clinicians (23% vs 46%)
Bateman & Fonagy	1999, 2008	MBT for BPD; 13% met criteria at 8 years vs 87%
Giesen-Bloo et al.	2006	ST recovery 45.5% vs TFP 24.4% for BPD
Bamelis et al.	2014	ST for Cluster C PDs: recovery 81% vs TAU 52%
Project MATCH	1997	TSF = CBT = MET for alcohol use disorder
TADS	2004	Adolescent depression: CBT + fluoxetine (71%) > fluoxetine (61%) > CBT (43%)
Mehlum et al.	2014	DBT-A reduced adolescent self-harm
Colom et al.	2003	Group psychoeducation reduced bipolar relapse at 5 years
Patel et al. (MANAS)	2010	Lay-delivered BA effective in India (Lancet)

EXAM PEARL

Minimum landmark trial set for viva: Rush (1977), Linehan (1991/2006), Bateman & Fonagy (1999), Giesen-Bloo (2006), Foa (2005), Teasdale (2000), Wampold (2001), Shedler (2010), Cuijpers (2019).

14.2 Key Meta-Analyses

Authors	Year	Focus	Main Result
Wampold	2001	Therapy vs no treatment	d = 0.80 overall
Cuijpers et al.	2008	Psychotherapy for depression	d = 0.67 (0.42 corrected for pub bias)
Shedler	2010	Psychodynamic therapy	d = 0.97; sleeper effect
Horvath et al.	2011	Alliance-outcome	r = 0.275
A-Tjak et al.	2015	ACT	g = 0.57 vs controls
Steinert et al.	2017	Psychodynamic vs CBT	g = -0.07 (no difference)
Cuijpers et al.	2019	Depression (network MA, 331 RCTs)	All 7 therapy types effective; combo > mono
Johnson et al.	1999	EFT-C	d = 1.30

15. HIGH-YIELD NUMBERS

15.1 Effect Sizes

Number	Meaning	Source
d = 0.80	Overall psychotherapy vs no treatment	Wampold (2001)
d = 0.00–0.20	Differences between bona fide therapies	Wampold (2001)
d = 0.97	Psychodynamic therapy	Shedler (2010)
d = 1.30	EFT for couples (largest for any couples therapy)	Johnson (1999)
d > 1.0	Clark's CT for social anxiety	Clark (2003)
d = 1.27	Clark's CT for PTSD	Ehlers & Clark (2005)
d = 0.33–0.44	CBTp vs TAU	Morrison (2014)
r = 0.275	Alliance-outcome correlation	Horvath (2011)

15.2 Key Percentages and Ratios

Number	Meaning	Source
40/30/15/15	Client/relationship/expectancy/technique (outcome variance)	Lambert (1992)
5–9%	Outcome variance explained by therapist effects	Wampold (2001)
5:1	Positive-to-negative ratio in stable couples	Gottman (1999)
93.6%	Gottman's divorce prediction accuracy	Gottman (1999)
31% vs 76%	Relapse: prior CT vs medication withdrawal	Hollon (2005)
23% vs 46%	Suicide attempts: DBT vs community experts	Linehan (2006)
37% vs 66%	Relapse: MBCT vs TAU (3+ episodes)	Teasdale (2000)
13% vs 87%	BPD criteria at 8 years: MBT vs control	Bateman & Fonagy (2008)
45.5% vs 24.4%	Recovery: ST vs TFP for BPD	Giesen-Bloo (2006)
81%	ST recovery for Cluster C PDs	Bamelis (2014)
70–73%	EFT-C couple recovery rate	Johnson (1999)
50%	Relapse reduction from family intervention in schizophrenia	Leff/Pilling (2000/2002)
60–70%	ERP response rate for OCD	Foa (2005)
80–90%	Panic-free rate with Clark's CBT	Clark (1994/1999)

Number	Meaning	Source
69%	Perpetual (unsolvable) couple problems	Gottman

15.3 Effect Size Interpretation

Cohen's d	NNT (approx.)	Label
0.2	~16	Small
0.5	~6	Medium
0.8	~4	Large
1.0+	~3	Very large

15.4 "Who Proved What" Quick Reference

- **CBT works for depression:** Rush 1977
- **CBT works for severe depression:** DeRubeis 2005
- **CBT has enduring effects:** Hollon 2005
- **All therapies roughly equivalent:** Cuijpers 2008/2019; Wampold 2001
- **MBCT prevents relapse (3+ episodes):** Teasdale 2000
- **DBT reduces self-harm:** Linehan 1991
- **DBT beats expert treatment:** Linehan 2006
- **MBT works (longest follow-up):** Bateman & Fonagy 1999/2008
- **ST has highest BPD recovery rate:** Giesen-Bloo 2006
- **TFP improves mentalisation:** Clarkin 2007
- **Psychodynamic therapy matches CBT:** Steinert 2017
- **Sleeper effect exists:** Shedler 2010
- **EFT-C has largest couples effect size:** Johnson 1999
- **Family intervention halves schizophrenia relapse:** Leff 2000

- **Four Horsemen predict divorce:** Gottman 1999
- **Alliance predicts outcome:** Bordin 1979; Horvath 2011
- **Rupture-repair is therapeutic:** Safran & Muran 2000
- **BA works for severe depression:** Dimidjian 2006

Final Integration: Key Themes Across All Modalities

1. The Relationship Is the Foundation

Every modality -- from CBT to psychoanalysis to DBT -- relies on the therapeutic relationship as a primary or necessary vehicle for change. Lambert's 30%, Bordin's alliance, Rogers' conditions, Linehan's dialectical stance, Young's limited reparenting, Fonagy's not-knowing curiosity -- all converge on the same truth.

2. All Effective Therapies Share Common Factors

Frank & Frank's four elements (relationship, setting, rationale, ritual) appear in every evidence-based therapy. The Dodo bird verdict (Luborsky 1975, Wampold 2001) holds for most conditions; exceptions exist for specific disorders (ERP for OCD, exposure for phobias).

3. Technique Matters for Specific Disorders

While common factors dominate overall, specific techniques have clear advantages: ERP for OCD, exposure for phobias, Clark's model for panic, interoceptive exposure for panic, DBT skills for BPD. Know the model, not just the brand.

4. Duration Follows Complexity

Axis I disorders: 12–20 sessions. Personality disorders: 1–3 years. Chronic/complex presentations: open-ended. The more characterological the problem, the longer the treatment.

5. Integration Is the Future

Third-wave therapies already integrate acceptance + change, mindfulness + behaviour, cognition + emotion. Schema Therapy integrates CBT + attachment + gestalt + psychodynamic. The field is moving toward unified protocols and personalised treatment selection.

> CLINICAL ANCHOR
>
> The single most useful clinical skill is not any specific technique -- it is the capacity to form a working alliance, maintain it through ruptures, and repair it when it breaks. This is what the evidence consistently shows predicts outcome.

Sprint Deep Study compiled from: Weave Psychotherapy Volumes 2–12. Synthesised for MD Psychiatry exam preparation.

Dr. Wilfred Dsouza | Weave -- Centre for Integrative Psychiatry

02

Clinical Quick Reference

Psychotherapy Sprint — Weave Psychotherapy Vol. 1

D6: Psychotherapy Sprint -- Ultimate Quick Reference

KEY INSIGHT

Purpose: Pure tables, mnemonics, and key facts -- the final cheat sheet before the exam

1. All Modalities at a Glance

Modality	Founder	Year	Core Mechanism	Best For	Duration
Psychoanalysis	Freud	1895	Insight via interpretation of unconscious	Character pathology	Years, 3–5×/week
Psychodynamic (LTPP)	Gabbard, Shedler	Modern	Interpretation + new relational experience	PDs, chronic depression	>24 sessions
Supportive therapy	Wolberg	1954	Bolster defences, restore functioning	Crisis, ego weakness, psychosis	Variable
CBT	Beck	1960s	Correct distorted cognitions	Depression, anxiety, OCD, PTSD	12–20 sessions
Behaviour therapy	Wolpe, Skinner	1950s	Conditioning/extinction	Phobias, OCD, depression (BA)	8–20 sessions
DBT	Linehan	1993	Skills + validation + dialectics	BPD, suicidality	1 year
Schema Therapy	Young	1990	Schema healing via limited reparenting + IR	BPD, chronic PDs	1–3 years
ACT	Hayes	1999	Psychological flexibility	Transdiagnostic	8–16 sessions

Modality	Founder	Year	Core Mechanism	Best For	Duration
MBCT	Segal et al.	2002	Doing mode -> Being mode	Depression relapse (3+ episodes)	8 weeks
IPT	Klerman	1984	Resolve interpersonal problem area	Depression, BN, perinatal	16 sessions
EMDR	Shapiro	1989	Adaptive information processing	PTSD	8–12 sessions
MBT	Bateman & Fonagy	1999	Restore mentalization	BPD	18 months
TFP	Kernberg	2006	Interpret split object relations	BPD	1–3 years, 2×/week
EFT-C	Johnson	1980s	Restructure attachment bond	Couples distress	8–20 sessions
MI	Miller & Rollnick	1983	Evoke patient's own change talk	Substance use, ambivalence	Brief
Gestalt	Perls	1960s	Awareness of contact + here-and-now	Unfinished business, splits	Variable
Logotherapy	Frankl	1946	Will to meaning	Existential vacuum	Variable

Modality	Founder	Year	Core Mechanism	Best For	Duration
TA	Berne	1961	Ego state analysis + game disruption	Interpersonal patterns	Variable

2. Defence Mechanisms Master Table (Condensed Vaillant)

Level	Defence	One-Line Definition
Primitive	Splitting	All-good/all-bad
Projective identification	Project + pressure other to enact	
Denial	Disregard reality	
Dissociation	Disrupted consciousness	
Idealisation/Devaluation	Extreme positive or negative attribution	
Acting out	Impulsive enactment	
Somatisation	Emotion -> body symptoms	
Regression	Return to earlier phase	
Neurotic	Repression	Expel from consciousness (internal)
Displacement	Shift to less threatening target	
Reaction formation	Wish -> opposite	
Intellectualisation	Abstract ideation avoids feeling	
Isolation of affect	Separate idea from emotion	
Rationalisation	Plausible untrue justification	
Undoing	Opposite action to negate	
Mature	Sublimation	Channel into valued activity
Humour	Comic distance without denial	
Suppression	Consciously postpone	

Level	Defence	One-Line Definition
Altruism	Service to others	
Anticipation	Realistic future planning	

Rule: Splitting-based = borderline. Repression-based = neurotic. Mature = healthy.

3. Cognitive Distortions Master Table

Distortion	Quick Definition
All-or-nothing	Black/white categories
Catastrophizing	Predict the worst
Disqualifying the positive	Dismiss positives
Emotional reasoning	Feelings = evidence
Labeling	Fixed global tag
Magnification/minimization	Inflate bad, shrink good
Mental filter	Focus on one negative
Mind reading	Assume others' thoughts
Overgeneralization	One event -> sweeping rule
Personalization	Others' behaviour = about me
Should statements	Rigid rules
Tunnel vision	See only negatives

4. DBT Skills + Mnemonics Master Table

Module	Skill/Mnemonic	Components
Mindfulness	Wise Mind	Emotional + Reasonable = Wise
"What" skills	Observe, Describe, Participate	
"How" skills	Non-judgmentally, One-mindfully, Effectively	
DT – Crisis	TIPP	Temperature, Intense exercise, Paced breathing, Progressive relaxation
STOP	Stop, Take a step back, Observe, Proceed mindfully	
ACCEPTS	Activities, Contributing, Comparisons, Emotions, Pushing away, Thoughts, Sensations	
IMPROVE	Imagery, Meaning, Prayer, Relaxation, One thing, Vacation, Encouragement	
DT – Accept	Radical Acceptance	Pain + Non-acceptance = Suffering
Turning the Mind	Ongoing choice to accept	
ER	ABC PLEASE	Accumulate positives, Build mastery, Cope ahead + PhysicaL illness, Eating, Avoid substances, Sleep, Exercise
Check the Facts	Is emotion fitting the facts?	
Opposite Action	Act opposite when emotion doesn't fit	

Module	Skill/Mnemonic	Components
IE	DEAR MAN	Describe, Express, Assert, Reinforce, Mindful, Appear confident, Negotiate
GIVE	Gentle, Interested, Validate, Easy manner	
FAST	Fair, no Apologies, Stick to values, Truthful	
THINK	Think, Have empathy, Interpretations, Notice, Kindness	

Validation levels: (1) Listen, (2) Reflect, (3) Mind-read, (4) Past history, (5) Current context (MOST IMPORTANT), (6) Radical genuineness (HIGHEST).

5. 18 Early Maladaptive Schemas Table

#	Domain	Schema	Core Belief
1	I. Disconnection (5)	Abandonment	"People will leave"
2	Mistrust/Abuse	"Others will hurt me"	
3	Emotional Deprivation	"Needs won't be met"	
4	Defectiveness	"I am flawed"	
5	Social Isolation	"I don't belong"	
6	II. Impaired Autonomy (4)	Dependence	"I can't handle life"
7	Vulnerability to Harm	"Catastrophe is coming"	
8	Enmeshment	"No identity apart from others"	
9	Failure	"I will always fail"	
10	III. Impaired Limits (2)	Entitlement	"Rules don't apply to me"
11	Insufficient Self-Control	"I can't control impulses"	
12	IV. Other-Directed (3)	Subjugation	"I must submit"
13	Self-Sacrifice	"Others' needs before mine"	
14	Approval-Seeking	"I need approval to feel worthy"	
15	V. Overvigilance (4)	Negativity/Pessimism	"Things will go wrong"
16	Emotional Inhibition	"I must suppress emotions"	

#	Domain	Schema	Core Belief
17	Unrelenting Standards	"I must be perfect"	
18	Punitiveness	"Mistakes deserve punishment"	

Domain distribution: 5–4–2–3–4. **Coping styles:** Surrender, Avoidance, Overcompensation. **BPD modes:** Abandoned child, Angry child, Punitive parent, Detached protector.

6. All Therapy Comparison Mega-Table

Dimension	Psychodynamic	CBT	DBT	Schema Therapy	ACT	IPT
Focus	Unconscious conflict	Distorted cognitions	Emotion dysregulation	Early schemas + modes	Experiential avoidance	Interpersonal problem area
Change mechanism	Insight + relationship	Restructuring + experiments	Skills + validation	IR + reparenting	Flexibility (hexaflex)	Resolve problem area
Therapist role	Participant-observer	Collaborative empiricist	Dialectical coach	Limited reparenter	Models flexibility	Active, supportive
Structure	Low (patient-led)	High (agenda, homework)	High (manual, diary cards)	Semi-structured	Moderate	Moderate
Homework	Rarely	Central	Diary cards	Schema diaries	Values exercises	Communication analysis
Duration	>6 months	12–20 sessions	1 year	1–3 years	8–16 sessions	16 sessions
Best for	PDs, chronic depression	Anxiety, depression, OCD	BPD, suicidality	BPD, chronic PDs	Transdiagnostic	Depression, BN
Effect size	0.97 (Shedler)	0.71–1.56 (varies)	Moderate-large (BPD)	45.5% recovery (BPD)	g = 0.57	0.67 (depression)

7. Disorder -> Therapy Master Table (32 Disorders)

Disorder	1st Line	Key Number
MDD (mild-mod)	CBT, BA, IPT	d = 0.71
MDD (severe)	CBT + SSRI	Combo > mono
Recurrent MDD (relapse)	MBCT	NNT = 4
Treatment-resistant MDD	CBT augmentation	Wiles 2013
GAD	CBT, Applied Relaxation	d = 0.80
Social anxiety	CBT (Clark & Wells)	d = 1.20
Specific phobia	In-vivo exposure	~90% response
Panic disorder	CBT (Clark)	80–90% panic-free
OCD	ERP	60–70% response
PTSD	TF-CBT, EMDR	d = 1.0–1.5
Complex PTSD	Phase-based (STAIR + PE)	Cloitre 2010
BPD	DBT, MBT	23% vs 46% (Linehan)
NPD	Schema Therapy	--
AvPD	CBT, Schema Therapy	81% recovery (Bamelis)
AN (adolescent)	FBT (Maudsley)	Lock 2010
AN (adult)	CBT-ED, MANTRA, SSCM	Zipfel 2014
BN	CBT-E	~50% remission
BED	CBT, self-help	Wilfley 2002
Alcohol use	MI, CBT, TSF	MATCH: all equivalent
Stimulant use	CBT, CM	d = 0.4–0.6
Schizophrenia	CBTp + family	d = 0.33–0.44

Disorder	1st Line	Key Number
Bipolar	Psychoeducation + stabiliser	Colom 2003
Insomnia	CBT-I	d = 0.98
Adolescent depression	CBT + SSRI	TADS: 71%
Adolescent self-harm	DBT-A	Mehlum 2014
Child anxiety	CBT (Coping Cat)	60–80% remission
Child conduct	Parent training	NICE CG158
Perinatal depression	CBT or IPT	O'Hara 2000
Couples distress	EFT-C	d = 1.30
Chronic depression	CBASP	Keller 2000
Hoarding	CBT + MI	Steketee 2007
BDD	CBT + SSRI	Veale 2010

8. Landmark Trials Key Numbers

Trial	Year	Key Number
Lambert	1992	40/30/15/15 outcome variance
Wampold	2001	d = 0.80 (therapy vs none); d = 0.00–0.20 (between therapies)
Rush et al.	1977	First CBT RCT -- CT > imipramine
Hollon et al.	2005	Relapse: CT 31% vs med withdrawal 76%
DeRubeis et al.	2005	CT = paroxetine (58% vs 58%) for severe MDD
Dimidjian et al.	2006	BA = medication > CT for severe depression
Teasdale et al.	2000	MBCT relapse: 37% vs 66% (3+ episodes)
Kuyken et al.	2015	MBCT = maintenance antidepressants
Clark et al.	2003	CBT for social anxiety d > 1.0 vs fluoxetine
Foa et al.	2005	ERP = clomipramine for OCD
Linehan et al.	2006	DBT: 23% vs 46% suicide attempts
Bateman & Fonagy	2008	MBT: 13% vs 87% still meet BPD criteria at 8 years
Giesen-Bloo et al.	2006	ST 45.5% vs TFP 24.4% recovery

Trial	Year	Key Number
Bamelis et al.	2014	ST 81% recovery for Cluster C PDs
Shedler	2010	Psychodynamic d = 0.97; sleeper effect
Gottman	1999	5:1 ratio; 93.6% divorce prediction
Johnson et al.	1999	EFT d = 1.30
TADS	2004	CBT+SSRI 71% > SSRI 61% > CBT 43%
Project MATCH	1997	TSF = CBT = MET (all equivalent)
Pilling et al.	2002	Family intervention halves schizophrenia relapse

9. All Mnemonics Collected

Mnemonic	Stands For	Context
PRESS	Placebo, Relationship, Emotional catharsis, Suggestion, Social dynamics	Wolberg's nonspecific factors
EUG / CUE	Empathy, UPR, Genuineness	Rogers' therapist conditions
CASE	Presenting, Recent, Past, Immediate	Suicide assessment (Shea)
SURMCRA	Subjectivity, Unconscious, Resistance, Multiple functions, Childhood, Response (CT), Attribution (T)	Gabbard's 7 principles
SPA-DIRSS	Splitting, PI, Acting out, Denial, Idealisation, Regression, Somatisation, Schizoid fantasy	Primitive defences
NTR	Narcissistic injury, Triumph over therapist, Refusal to gratify	Negative therapeutic reaction
Three H's	Helplessness, Heartless world, Hopeless character	Negative core beliefs
FLAME MOST	Fortune-telling, Labeling, All-or-nothing, Mind reading, Emotional reasoning, Magnification, Overgeneralization, Should, Tunnel vision	Cognitive distortions
MADAM-SURF	Mood, Agenda, Discuss, Agenda prioritise, Middle -- Summarise, Update, Review, Finish	CBT session structure

Mnemonic	Stands For	Context
TRAP-TRAC	Trigger, Response, Avoidance Pattern -> Alternative Coping	Behavioural activation
TIPP	Temperature, Intense exercise, Paced breathing, Progressive relaxation	DBT crisis survival
STOP	Stop, Take a step back, Observe, Proceed mindfully	DBT impulse control
ACCEPTS	Activities, Contributing, Comparisons, Emotions, Pushing away, Thoughts, Sensations	DBT distraction
IMPROVE	Imagery, Meaning, Prayer, Relaxation, One thing, Vacation, Encouragement	DBT coping
ABC PLEASE	Accumulate, Build mastery, Cope ahead + Physical, Eating, Avoid, Sleep, Exercise	DBT emotion regulation
DEAR MAN	Describe, Express, Assert, Reinforce, Mindful, Appear confident, Negotiate	DBT getting what you want
GIVE	Gentle, Interested, Validate, Easy manner	DBT maintaining relationship
FAST	Fair, no Apologies, Stick to values, Truthful	DBT self-respect
THINK	Think, Have empathy, Interpretations, Notice, Kindness	Adolescent DBT
PACE	Partnership, Acceptance, Compassion, Evocation	MI spirit

Mnemonic	Stands For	Context
OARS	Open questions, Affirmations, Reflections, Summaries	MI core skills
DARN-CAT	Desire, Ability, Reasons, Need -- Commitment, Activation, Taking steps	Change talk
DFIM	Death, Freedom, Isolation, Meaninglessness	Yalom's 4 concerns
I-PRDC	Introjection, Projection, Retroflection, Deflection, Confluence	Gestalt contact disturbances
FEAR	Feeling, Expecting bad, Actions, Results	Coping Cat (child anxiety)
CRIS	Cohort, Role transitions, Intergenerational, Socio-cultural	Laidlaw's CCCF
CHE	Critical comments, Hostility, Emotional overinvolvement	Expressed emotion
C-C-D-S	Criticise, Condemn, Defend, Shut down	Gottman's Four Horsemen

10. Gottman Four Horsemen + Antidotes

Horseman	What It Is	Antidote
Criticism	Global character attack	**Gentle start-up:** "I feel [X] about [Y]. I need [Z]."
Contempt	Superiority + disgust (WORST predictor)	**Fondness & admiration:** daily appreciation
Defensiveness	Counter-blame, victim stance	**Accept responsibility:** "You have a point"
Stonewalling	Emotional shutdown (85% male)	**Self-soothing:** 20+ min break when HR > 100 bpm

Key: 5:1 positive-to-negative ratio. Contempt = #1 divorce predictor. 69% of problems are perpetual.

Sound Relationship House (bottom to top)

Level	Component	One-Line
1 (Base)	Love Maps	Cognitive map of partner's inner world
2	Fondness & Admiration	Respect; antidote to contempt
3	Turning Toward	Respond to bids for connection
4	Positive Sentiment Override	Benefit of the doubt (from strong friendship)
5	Manage Conflict	Solvable (repair) + perpetual (dialogue)
6	Make Life Dreams Come True	Honour aspirations; overcome gridlock
7 (Top)	Create Shared Meaning	"Culture of two"
WALLS	Trust & Commitment	Weight-bearing walls

11. Yalom's 11 Therapeutic Factors

#	Factor	One-Line
1	Instillation of hope	Seeing others recover
2	Universality	"I am not alone"
3	Imparting information	Psychoeducation
4	Altruism	Helping others heals
5	Corrective recapitulation	Rework family dynamics
6	Socialising techniques	Learn skills via feedback
7	Imitative behaviour	Model after others
8	**Interpersonal learning**	**Group as social microcosm** (most distinctive)
9	**Group cohesiveness**	**Belonging** (most foundational = alliance equivalent)
10	Catharsis	Safe emotional release
11	Existential factors	Death, meaning, isolation

12. Top 30 Viva Questions with One-Line Answers

#	Question	One-Line Answer
1	What are the common factors in psychotherapy?	Lambert 40/30/15/15: client, relationship, expectancy, technique. Relationship > technique.
2	What is Bordin's working alliance?	Three components: goals, tasks, bond. r = 0.275 with outcome.
3	Name Rogers' core conditions.	EUG: Empathy, UPR, Genuineness. Low EUG = patient deterioration (Truax).
4	What is the Dodo bird verdict?	Luborsky 1975: bona fide therapies produce roughly equivalent outcomes.
5	Describe Lambert's outcome factors.	40% client, 30% relationship, 15% expectancy, 15% technique.
6	What is the CASE approach?	Shea: Chronological suicide assessment -- presenting, recent, past, immediate events.
7	Compare supportive vs expressive therapy.	Supportive: bolster defences, foster + transference. Expressive: interpret defences, analyse transference.
8	What is the sleeper effect?	Shedler 2010: psychodynamic therapy effect sizes INCREASE at follow-up.

#	Question	One-Line Answer
9	Explain projective identification.	Project + interpersonal pressure + target enacts the projected role. Three steps.
10	What is Beck's cognitive triad?	Negative views of self, world, future. Maintained by schema-driven information processing bias.
11	Name 5 cognitive distortions.	All-or-nothing, catastrophizing, emotional reasoning, mind reading, should statements.
12	What is collaborative empiricism?	CBT stance: therapist + client as co-investigators testing hypotheses about cognitions.
13	Compare classical and operant conditioning.	Classical: stimulus pairing, involuntary. Operant: consequence-based, voluntary.
14	What is the DBT biosocial model?	Biology (sensitivity + reactivity + slow return) x invalidating environment = dysregulation.
15	Name the DBT target hierarchy.	(1) Life-threatening, (2) therapy-interfering, (3) quality-of-life, (4) skills.
16	What are the six DBT validation levels?	Listen, reflect, mind-read, past history, current context (MOST IMPORTANT), radical genuineness.
17	List the 5 schema domains.	Disconnection, Impaired Autonomy, Impaired Limits, Other-Directedness, Overvigilance. 5–4–2–3–4.

#	Question	One-Line Answer
18	What is imagery rescripting?	Access memory via affect bridge; helping figure changes outcome. NOT exposure -- memory is rescripted.
19	Describe the ACT hexaflex.	6 processes: acceptance, defusion, present moment, self-as-context, values, committed action.
20	When does MBCT work?	Only for 3+ depressive episodes (differential activation). No benefit for 1–2 episodes.
21	What are Yalom's 4 ultimate concerns?	Death, freedom, isolation, meaninglessness (DFIM).
22	Name the 4 IPT problem areas.	Grief, role disputes, role transitions, interpersonal deficits.
23	What are the 8 EMDR phases?	History, preparation, assessment, desensitization, installation, body scan, closure, re-evaluation.
24	Compare DBT, MBT, TFP, ST for BPD.	DBT: skills. MBT: mentalization. TFP: transference interpretation. ST: IR + reparenting. All effective.
25	What is Gottman's strongest divorce predictor?	Contempt (superiority + disgust). Four Horsemen predict at 82–93% accuracy.

#	Question	One-Line Answer
26	What is group cohesiveness?	Yalom: equivalent of therapeutic alliance in group therapy. Most foundational factor.
27	Name 3 family therapy schools + founder.	Structural (Minuchin), Strategic (Haley), Bowenian (Bowen).
28	What is expressed emotion?	Critical comments + hostility + EOI. High EE = ~50% schizophrenia relapse at 9 months.
29	What therapy for adolescent AN?	FBT (Maudsley model). Parents take control of feeding. Lock 2010.
30	What is the evidence for psychodynamic therapy?	Shedler 2010: d = 0.97. Steinert 2017: equivalent to CBT (g = -0.07). Sleeper effect unique.

13. Family Therapy Schools -- Quick Card

School	Founder	One Technique
Structural	Minuchin	Enactment, restructuring
Strategic	Haley/MRI	Directives, paradox
Bowenian	Bowen	Genogram, differentiation
Systemic/Milan	Selvini Palazzoli	Circular questioning
Experiential	Satir, Whitaker	Family sculpting
Narrative	White & Epston	Externalising the problem
Solution-Focused	de Shazer, Berg	Miracle question

Key concepts: Homeostasis (system resists change), triangulation (Bowen), enmeshment vs disengagement (Minuchin), double bind (Bateson), identified patient, circular causality.

14. BPD Treatments -- Quick Comparison

Feature	DBT	MBT	TFP	Schema Therapy
Target	Emotion dysregulation	Mentalizing failure	Split object relations	EMSs + modes
Stance	Dialectical	Not-knowing	Neutral	Limited reparenting
Key technique	Chain analysis + skills	Stop-and-rewind	Transference interpretation	IR + chair work
RCT	Linehan 2006	Bateman 2008	Clarkin 2007	Giesen-Bloo 2006
Dropout	~25%	~25%	~50%	~27%
Recovery	Reduced self-harm	13% meet BPD at 8yr	Improved RF + attachment	45.5%
NICE	Yes	Yes	No	No

15. Shea's Validity Techniques -- Quick Card

Technique	One-Line
Behavioural incident	"Verbal videotape" of specific facts
Shame attenuation	Frame positive answer as ego-syntonic
Gentle assumption	Assume the behaviour is occurring
Symptom amplification	Set the upper limit high
Denial of the specific	Ask about specific substances/methods
Normalisation	"Many people in your situation..."

16. Key Therapy Acronyms

Acronym	Full Name
BA	Behavioural Activation
CBT-E	Enhanced CBT (eating disorders)
CBT-I	CBT for Insomnia
CBTp	CBT for Psychosis
CM	Contingency Management
CBASP	Cognitive Behavioural Analysis System of Psychotherapy
CPT	Cognitive Processing Therapy
CT-R	Recovery-Oriented Cognitive Therapy
ERP	Exposure and Response Prevention
FBT	Family-Based Treatment (Maudsley)
FFT	Family-Focused Therapy (bipolar)
IBCT	Integrative Behavioural Couples Therapy
IPSRT	Interpersonal and Social Rhythm Therapy
MANTRA	Maudsley AN Treatment for Adults
MST	Multisystemic Therapy
NET	Narrative Exposure Therapy
PCIT	Parent-Child Interaction Therapy
PE	Prolonged Exposure
SSCM	Specialist Supportive Clinical Management
STAIR	Skills Training in Affective and Interpersonal Regulation
TSF	Twelve-Step Facilitation

17. Effect Size Interpretation

Cohen's d	NNT (approx.)	Label
0.2	~16	Small
0.5	~6	Medium
0.8	~4	Large
1.0+	~3	Very large

Rule of thumb: d = 0.80 is the overall effect of psychotherapy vs no treatment (Wampold 2001). Anything above 1.0 is exceptional.

Sprint Quick Reference compiled from Weave Psychotherapy Volumes 2–12.

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